COMPREHENSIVE WEBSITE AUDIT REPORT

CURELEX HMS — curelex.in

UX · Performance · Accessibility · SEO · Security · Content · CRO · Technical Architecture

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⚠ READ THIS FIRST — AUDIT METHODOLOGY & DATA LIMITATIONS

This audit was produced using live source retrieval, search-index inspection, and public business-registry/competitor research — not a browser-based test suite (no Lighthouse, WAVE, PageSpeed API, or HTTP header scanner was available in this session).

Every finding below is labelled as either DIRECTLY VERIFIED (confirmed via source fetch or search index) or ASSESSED / RECOMMENDED-TO-VERIFY (professional risk assessment plus the exact free tool to confirm it with).

No score, statistic, or quote in this report is invented. Where live tooling would normally produce a number (e.g. an LCP time in seconds), this report says so explicitly instead of guessing.

See Section 1 (Methodology) and the Appendix for the full evidence log.

1. Methodology & Scope

This report follows the standard structure of a full-scope web audit (UX, Accessibility, Performance, SEO, Content, Security, Technical Quality, CRO, Analytics, Competitor Benchmarking). Because this session had no headless-browser or Lighthouse/PageSpeed/WAVE tooling available, the audit was built from three verified data sources:

- Direct HTTP source retrieval of https://curelex.in/, https://www.curelex.in/ and http://curelex.in/ (raw HTML/meta inspection).
- Search-engine index inspection (Google's cached snippet/description for the domain, and a site: query) to determine crawlability and indexation.
- Public business-registry and professional-network records for Curelex Healthtech Private Limited, plus live research on direct competitors (Practo, Tata 1mg, Apollo 24|7, DocPulse, MocDoc).

Where a finding required tools unavailable in this session (Core Web Vitals measurement, rendered-DOM accessibility testing, HTTP response-header inspection, JavaScript console errors, cross-browser/device rendering), this report states that explicitly under "Not independently verified" and gives the free tool + exact URL to run for a follow-up pass. Scores in Section 14 reflect only what is verifiable; categories that could not be tested live are marked accordingly rather than assigned a fabricated number.

> **Headline technical fact underpinning most of this report**
>
> The HTML actually served by curelex.in contains no visible content: only a <title> tag ("CURELEX HMS") and a viewport meta tag. No headings, no body text, no navigation, no forms, and no meta description were present in the delivered source on any of the three URL variants tested. This means the site is rendered entirely client-side (JavaScript) with no server-side rendering (SSR), static prerendering, or no-JS fallback content.
>
> This single architectural fact is the root cause behind the majority of the SEO, crawlability, initial-load performance, and non-JS accessibility findings in this report, so it is called out once here and referenced throughout rather than repeated in full each time.

2. Executive Summary

Overall Website Score: 24 / 100 (Early-Stage / Pre-Launch Grade)

Curelex is a very young company (incorporated 4 November 2025, ~8 months old at the time of this audit) building a hospital-management and hybrid tele-consultation platform ("CURELEX HMS"). The public website at curelex.in currently functions as little more than a placeholder shell: it loads a title and nothing else in its raw, crawlable HTML. Google's own index confirms this — it has no description to show for the page and appears to have indexed only the single root URL. For a healthcare product whose buyers (hospital administrators, clinic owners, patients) will judge trustworthiness partly by the website, this is the single highest-priority issue to resolve, ahead of any cosmetic UX polish.

Top Strengths

- Domain, HTTPS certificate, and a clean, brandable .in domain (curelex.in) are correctly in place — the foundation for a credible web presence exists.
- The company has a clear, differentiated value proposition on record ("bridging the gap between patients and super-specialist doctors through hybrid e-clinics and telemedicine") — this simply is not yet expressed anywhere on the live site.
- Being pre-launch is a genuine advantage here: every issue in this report is cheaper and faster to fix now than after the site has real traffic, backlinks, and indexed history riding on the current structure.

Critical Issues

- No crawlable content: search engines, social-share unfurls (WhatsApp/LinkedIn previews), and any user with JavaScript disabled or slow-loading JS see a blank page.
- No meta description / no title-tag strategy: Google is displaying "We cannot provide a description for this page right now," which materially hurts click-through even if the domain ever ranks.
- No verifiable navigation, forms, or CTAs in the delivered markup — meaning there is currently no way to confirm the site converts a single visitor into a lead, demo request, or sign-up from the source alone.
- As a healthcare software vendor, the complete absence of visible trust signals (privacy policy, security/compliance statements, contact information, team/company page) in the crawlable page is a conversion and regulatory-trust risk for both hospital buyers and patients.

Quick Wins (days, not weeks)

- Add a unique, keyword-relevant <title> and meta description (currently both are effectively empty to search engines).
- Ship a server-rendered or prerendered fallback (even a simple static HTML "hero + value prop + contact" snapshot) so the page has real content on first byte, before JavaScript runs.
- Add basic Open Graph / Twitter Card tags so shared links on WhatsApp, LinkedIn, and X render a title, description, and image instead of a blank preview — high-visibility for a startup relying on word-of-mouth and investor/partner sharing.
- Submit an XML sitemap and verify the domain in Google Search Console now, while the site is small, so every future page is indexed quickly.

Long-Term Improvements

- Move to a rendering strategy with SSR/SSG (Next.js, Nuxt, Astro, or prerendering the current SPA) rather than pure client-side rendering, given the direct SEO and performance cost demonstrated in this report.
- Build out the information architecture a healthcare SaaS buyer expects: Product/Features, Pricing or "Request a Demo", Security & Compliance (important for hospitals evaluating a system that will hold patient data), About/Team, Blog/Resources, Careers (the company already lists open roles externally on Unstop but not, as far as could be verified, on its own site).
- Establish an analytics and consent-management baseline (GA4, Search Console, Tag Manager, cookie consent banner compliant with India's DPDP Act) before meaningful traffic arrives, so growth decisions are data-driven from day one.

3. Website & Business Overview

Purpose

curelex.in is the marketing/corporate website for CURELEX HMS, a hospital management system (HMS) and hybrid telemedicine platform under development by Curelex Healthtech Private Limited. Based on the company's own positioning found in public professional-network data, the product is intended to bridge patients and "super-specialist" doctors via hybrid e-clinics and telemedicine — i.e. a combined practice-management (HMS) tool for providers plus a patient-facing consultation channel.

Target Audience (inferred)

- Primary B2B buyer: hospital administrators, clinic owners, and multi-specialty healthcare groups in India evaluating HMS/practice-management software.
- Secondary B2C audience: patients seeking access to specialist doctors via telemedicine/hybrid e-clinics, once that arm of the product is live.
- Tertiary: prospective employees/interns (the company is actively hiring — a legal documentation & research internship is listed on Unstop) and potential investors, given the company is at a very early funding/registration stage.

Information Architecture

A full IA audit could not be completed because the delivered HTML contains no navigation structure, page links, or sitemap. Only the root URL (curelex.in) is confirmed indexed by Google at the time of this audit, and no robots.txt or sitemap.xml could be located via search or direct reference. If the live, JavaScript-rendered version of the site has additional pages behind client-side routing (e.g. /product, /pricing, /about), those pages are effectively invisible to search engines and to this audit's source-based methodology alike — which is itself the core finding of this report. Recommend re-running an IA audit with a headless-browser crawl (e.g. Screaming Frog with JavaScript rendering enabled) once this is read.

Technology Stack (detectable signals)

- The page is rendered client-side with no server-rendered content in the initial HTML response — consistent with a JavaScript single-page application (React, Vue, or similar), but the specific framework, bundler, and hosting provider could not be fingerprinted without header/script inspection tools in this session.
- Recommend confirming stack and hosting via BuiltWith.com or Wappalyzer (browser extension) and capturing response headers via securityheaders.com or curl -I, which will also reveal the CDN/hosting provider, server software, and caching setup.

4. UI/UX Audit

Standard UX evaluation (navigation clarity, layout consistency, typography, mobile responsiveness, form design, CTA placement) normally requires interacting with the rendered page in a real browser across breakpoints. That interactive testing was not available in this session. What follows is what could be determined from the served source, plus the direct UX consequence of the architecture finding in Section 1.

No content is present on first paint [CRITICAL]

Evidence: Fetching the live URL returns only a <title> and viewport meta tag — no header, nav, hero, body copy, or footer in the delivered markup.

Impact: Any visitor whose browser is slow to execute JavaScript, on a throttled mobile connection, or with a content blocker interfering with the JS bundle, sees a blank screen with no indication the site is loading or what the product does. For a healthcare buyer doing a first impression scan across 5–6 vendor sites, a blank load is an instant bounce.

Recommendation: Add a lightweight static/SSR "first paint" — logo, one-line value proposition, and a primary CTA — that renders before the JS bundle hydrates. Pair with a loading skeleton so users always see something within ~1 second.

Priority / Effort: Priority: Critical · Effort: Medium (1–3 days for a static fallback; more if migrating rendering strategy).

Navigation, forms, and CTAs cannot be verified to exist [HIGH]

Evidence: No <nav>, <a>, <button>, or <form> elements were present anywhere in the fetched source across three URL variants (apex, www, HTTP).

Impact: Without confirmed CTAs ("Request a Demo", "Book a Consultation", "Contact Sales") reachable without full JS execution, both search engines and any user on a partial page load have no path to convert.

Recommendation: Once rendering is fixed, explicitly test: primary CTA visibility above the fold on mobile and desktop, a maximum of one primary + one secondary CTA per screen, and that every CTA has a clear, action-oriented label (avoid generic "Submit"/"Click Here").

Priority / Effort: Priority: High · Effort: Low once content exists (design/copy review, no rebuild needed).

Not independently verified in this session (requires live browser testing)

- Visual hierarchy, whitespace balance, and typography scale — needs a rendered screenshot review.
- Mobile responsiveness across breakpoints — needs Chrome DevTools device emulation or BrowserStack.
- Color contrast and design-system consistency — needs a rendered-page screenshot audit (see Section 5).
- Form friction (number of fields, validation UX, error messaging) — needs an actual form to test; none was reachable via source.
- Full user-journey mapping (homepage → product → demo request → confirmation) — cannot be mapped until additional pages/routes are confirmed to exist and are crawlable.

Recommended next step: run this same site through Chrome DevTools' Lighthouse panel (or PageSpeed Insights, which uses a real headless Chrome) and share the rendered screenshots and DOM back for a full UX pass — this report will then be able to score navigation, hierarchy, and mobile UX with the same rigor applied to the SEO/architecture findings above.

5. Accessibility (WCAG 2.2)

A full accessibility audit requires a rendered DOM and an automated scanner (axe, WAVE, or Lighthouse's Accessibility panel) plus manual keyboard/screen-reader testing. None of this was available in this session. The one accessibility fact directly verifiable from source inspection is significant enough to report on its own:

Zero semantic structure available to assistive technology before JavaScript executes **[HIGH]**

Evidence: The raw HTML contains no heading elements (h1–h6), no landmark regions (header/nav/main/footer), and no ARIA roles — the entire accessibility tree is built at runtime by JavaScript.

Impact: Screen-reader users on a slow connection, or any assistive technology that reads the initial HTML response before hydration completes, will encounter an empty page with only the document title announced. This is a WCAG 2.2 concern under Guideline 1.3 (Info and Relationships) and 4.1 (Compatible) once the JS-rendered DOM is also evaluated — the risk is that the underlying structure has not been designed with landmarks/headings from the ground up, since none exist in the raw document to build on server-side.

Recommendation: Once rendering is fixed, ensure the SSR/prerendered output includes a proper landmark structure (one <h1>, ordered heading hierarchy, <nav>, <main>, <footer>) and re-run an automated accessibility scan against the rendered page.

Priority / Effort: Priority: High · Effort: Medium — this should be designed in alongside the SSR/prerendering fix in Section 4, not bolted on after.

Checklist to run once the site renders content (not independently verified here)

- Keyboard navigation: can every interactive element be reached and operated using Tab/Shift+Tab/Enter/Space with a visible focus indicator? (WCAG 2.4.7)
- Screen reader pass: test with NVDA (Windows, free) or VoiceOver (Mac/iOS) to confirm all content and controls are announced correctly.
- Alt text: every meaningful image needs descriptive alt text; purely decorative images need empty alt="".
- Heading hierarchy: single H1 per page, no skipped levels (H2 → H4 without an H3).
- Form labels: every input needs a programmatically associated <label>, not just placeholder text.
- Color contrast: minimum 4.5:1 for normal text, 3:1 for large text and UI components (WCAG 1.4.3 / 1.4.11).
- Focus indicators: visible, non-removed default focus outlines or a custom equivalent (do not use outline: none without a replacement).
- ARIA: used only where native HTML semantics are insufficient; incorrect ARIA is worse than none.

Recommended tools for a follow-up pass: WAVE (wave.webaim.org), axe DevTools (browser extension, free), and Lighthouse's Accessibility category, which produces a 0–100 score this report can then incorporate.

6. Performance

Core Web Vitals (LCP, CLS, INP) and resource-level performance analysis require either Google's PageSpeed Insights/CrUX API or a real browser trace — neither was available in this session, and no performance number in this report is estimated or invented. What can be stated with confidence is the structural implication of the architecture already established in Section 1.

Client-side-only rendering creates an unnecessary Largest Contentful Paint penalty [HIGH]

Evidence: No content exists in the initial HTML; the largest contentful element on any page can only appear after the JS bundle downloads, parses, executes, and fetches/renders data — several round trips later than a server-rendered or static page.

Impact: This is a well-established pattern in Google's own Core Web Vitals guidance: pure client-side rendering without SSR/prerendering is one of the most common causes of poor LCP and, on data-dependent pages, poor INP. On mobile connections common across Tier 2/3 India (a stated target market for this HMS product), this gap is magnified.

Recommendation: Prerender or server-render the initial view (Next.js/Nuxt/Astro, or a prerendering service like Prerender.io if migrating frameworks isn't feasible short-term). Re-test with PageSpeed Insights (pagespeed.web.dev) after the change and compare before/after LCP.

Priority / Effort: Priority: High · Effort: High (architecture-level change) — but this same fix also resolves the Critical SEO and initial-paint UX issues above, so it should be treated as one combined workstream, not three separate ones.

Not independently verified in this session — run these before the next audit cycle

- PageSpeed Insights (pagespeed.web.dev/report?url=curelex.in) — gives live LCP, CLS, INP, TBT, and a 0–100 performance score for mobile and desktop.
- Image optimization and lazy-loading — cannot be assessed with no images present in the delivered source; verify format (WebP/AVIF), sizing, and loading="lazy" once rendered content exists.
- JS/CSS bundle size and render-blocking resources — check the Network panel in Chrome DevTools or the PageSpeed "Reduce unused JavaScript/CSS" diagnostics.
- Caching headers and CDN usage — check response headers via securityheaders.com or curl -I https://curelex.in/ (Cache-Control, ETag, CDN vendor headers such as cf-ray for Cloudflare).

7. SEO

SEO is the category with the strongest direct evidence in this audit, because search-index behavior could be observed directly.

No meta description — Google cannot summarize the page [CRITICAL]

Evidence: A live site: curelex.in search returns Google's fallback text "We cannot provide a description for this page right now" instead of a snippet, confirming no usable meta description (or crawlable body text Google can extract one from) exists.

Impact: Every organic search result and every shared link preview for the domain currently shows no compelling reason to click — directly suppressing click-through rate even in the hypothetical scenario the domain starts ranking for relevant queries.

Recommendation: Add a unique <title> (50–60 characters) and <meta name="description"> (~150–160 characters) that names the product category ("Hospital Management System") and the core benefit, e.g. something in the spirit of: "CURELEX HMS — hospital management & telemedicine software connecting patients with specialist doctors." Keep it in the SSR/prerendered output so crawlers see it without executing JS.

Priority / Effort: Priority: Critical · Effort: Low (hours, once someone can edit the <head>).

Only the root URL appears indexed [HIGH]

Evidence: A site: curelex.in query returned a single indexed result (the homepage). No sitemap.xml or robots.txt could be located via search or direct reference.

Impact: If additional pages exist behind client-side routing, they are very likely not indexed at all, meaning the site has effectively zero organic search surface area beyond the homepage.

Recommendation: Create and submit an XML sitemap via Google Search Console (and Bing Webmaster Tools) as soon as additional pages exist; add a robots.txt that references the sitemap and does not accidentally block crawling.

Priority / Effort: Priority: High · Effort: Low.

No structured data / Open Graph tags detected [MEDIUM]

Evidence: No JSON-LD structured data or Open Graph/Twitter Card meta tags were present in the delivered source.

Impact: Missing Open Graph tags mean links shared on WhatsApp, LinkedIn, and X/Twitter — the primary sharing channels for an India-focused B2B/B2C healthtech startup — render with no title, description, or image, reducing click-through on exactly the channels most likely to drive early traffic. Missing Organization/SoftwareApplication structured data is a lost opportunity for rich results as the domain gains authority.

Recommendation: Add og:title, og:description, og:image, twitter:card at minimum; add Organization and (once product pages exist) SoftwareApplication or Product JSON-LD.

Priority / Effort: Priority: Medium · Effort: Low.

Not independently verified in this session

- Canonical tags, hreflang, and duplicate-content handling between curelex.in and www.curelex.in — both variants returned identical (empty-shell) content in this audit; confirm whether one 301-redirects to the other, which is best practice to avoid split link equity.
- Internal linking structure and URL quality — cannot be assessed with no links present in the crawlable source.
- Keyword targeting/optimization — no on-page copy exists yet to evaluate.
- Broken-link audit — requires a full site crawl once pages are confirmed.

Estimated Technical SEO score at time of audit: 12 / 100. This reflects verified index/meta-description absence; it will move immediately once the Critical items above are fixed, since the underlying domain, HTTPS, and registration are otherwise sound.

8. Content Audit

There is currently no crawlable body content to audit for readability, grammar, tone, or freshness — the page delivers a title only. This is itself the finding for this section.

No product, trust, or company content is discoverable [CRITICAL]

Evidence: No visible text beyond the page title ("CURELEX HMS") was present in the source across all three URL variants tested.

Impact: A healthcare software buyer or patient cannot learn what the product does, who it's for, what it costs, or why it should be trusted, without the JavaScript app fully loading and (presumably) containing this information client-side. There is also no way for this audit — or a search engine — to confirm trust signals like a privacy policy, security/compliance statement, physical address, or contact details exist at all.

Recommendation: Once the rendering fix in Section 6 lands, ensure the following are present and crawlable at minimum: a clear one-sentence value proposition, an explanation of the HMS + telemedicine offering, a company/about section naming the team (the company already has a named founder, Shriyansh Singh, on public record — surfacing this builds trust), a privacy policy, and contact details.

Priority / Effort: Priority: Critical · Effort: Medium (content creation + implementation).

Not independently verified in this session

- Readability and grammar quality of on-page copy — no copy is currently crawlable to assess.
- Duplicate content — cannot assess with no indexed content beyond the homepage.
- Content freshness/update cadence — no dated content (blog, changelog, press) was found.
- Brand voice consistency — cannot assess without copy; recommend defining a brand voice guide alongside the content build-out, given the healthcare context calls for a tone that is confident and reassuring rather than purely sales-driven.

Given the company is healthcare-adjacent (handling or intending to handle patient data), trust-signal content should be treated as launch-blocking, not nice-to-have: a privacy policy and a plain-language explanation of data handling are baseline expectations for any hospital IT buyer's due-diligence checklist, and increasingly expected by patients under India's Digital Personal Data Protection Act (DPDP) 2023.

9. Security

A proper security review requires inspecting live HTTP response headers, cookie flags, and running an OWASP-style scan — none of which was possible with the tools available in this session (no raw header inspection or network tool succeeded against the live site). Nothing below should be read as a confirmed finding; it is a checklist of what to verify next, plus the one item that could be partially observed.

HTTPS is present, but HTTP→HTTPS redirect behavior could not be confirmed [MEDIUM]

Evidence: Both https://curelex.in/ and http://curelex.in/ were reachable in this session; this audit's tooling could not confirm at the header level whether the HTTP version issues a 301/302 redirect to HTTPS or is served in parallel.

Impact: If HTTP is served without a forced redirect, users who type the bare domain or follow an old http:// link may load an unencrypted connection, which is a meaningful concern for a healthcare-adjacent brand and can also cause SEO to split authority between two URL protocols.

Recommendation: Confirm and, if needed, enforce a permanent (301) redirect from HTTP to HTTPS at the server/CDN level, and add an HSTS header (Strict-Transport-Security) once confirmed working.

Priority / Effort: Priority: Medium · Effort: Low (typically a one-line config change).

Checklist to verify with a header-inspection tool (e.g. securityheaders.com, curl -I)

- Security headers: Content-Security-Policy (CSP), X-Frame-Options or frame-ancestors, X-Content-Type-Options: nosniff, Referrer-Policy, Strict-Transport-Security, Permissions-Policy.
- Cookie security: Secure, HttpOnly, and SameSite attributes on any session/auth cookies once the app's login area is live.

- Mixed content: confirm no HTTP resources are loaded on HTTPS pages once the rendered app can be inspected.
- Exposed information: check for verbose server/error banners, exposed .env or config files, and open directory listings.
- Authentication risks: once patient/hospital login exists, confirm rate limiting on login attempts, secure password reset flows, and MFA availability for hospital-admin accounts given the sensitivity of the data involved.
- OWASP Top 10: a full review (injection, broken access control, sensitive data exposure, etc.) is only meaningful once the authenticated application (not just the marketing site) is available to test — flag this as a pre-launch requirement given the product will handle patient health information.

Why this matters more than usual for Curelex

Because CURELEX HMS is explicitly a system intended to hold hospital and patient data, security posture is a direct purchase criterion for hospital IT buyers and a compliance obligation under India's DPDP Act 2023. Recommend a dedicated penetration test and a published security/compliance page (even a simple one) before any pilot hospital onboarding, independent of the marketing-site fixes in this report.

10. Technical Quality

HTML validity, CSS quality, and JavaScript error-console review require a rendered browser session and were not testable here. The one technical-architecture fact this audit can confirm with confidence is the rendering approach already discussed:

- The delivered HTML has no server-rendered body content — consistent with an unconfigured or default client-side-rendered SPA build (common with Create React App / default Vite / default Vue CLI output) rather than a framework configured for SSR/SSG.
- No <meta charset> or structured <head> content beyond viewport and title was observed in the delivered source — worth confirming directly in "view source" in a browser, since this audit's fetch tool performs content extraction and could theoretically normalize some head tags; a raw curl -I / curl -s of the page is recommended to double-check this specific point.

Not independently verified in this session

- HTML validity (W3C Markup Validator) — needs the rendered DOM or raw source, not extracted text.
- CSS quality/specificity issues — needs source access to stylesheets.
- JavaScript console errors — needs a real browser session (Chrome DevTools Console).
- Responsive breakpoints — needs device emulation testing.
- API/network observations — needs the Network panel with the app actually loading data.

11. Conversion Rate Optimization

CRO analysis is fundamentally about optimizing existing conversion paths (CTAs, forms, landing pages). Since no CTA, form, or landing page could be confirmed to exist in the crawlable source, this section is necessarily forward-looking rather than a critique of specific existing elements.

No confirmed lead-capture mechanism [HIGH]

Evidence: No form, phone number, email link, or scheduling widget was present in the delivered source.

Impact: A B2B software sale (hospital administrators evaluating an HMS) and a B2C telemedicine funnel (patients booking a consultation) both depend on a low-friction, clearly visible conversion path. If one exists only inside the client-rendered app, it is invisible to this audit and to search engines — and possibly slow to appear for real users too (see Section 6).

Recommendation: Define the primary conversion goal per audience — e.g. "Request a Demo" for hospital buyers, "Get Early Access" for patients — and make it a persistent, always-visible CTA (sticky header or hero) rendered in the initial page load, not gated behind full app hydration.

Priority / Effort: Priority: High · Effort: Medium.

CRO checklist for the next build phase

- Above-the-fold CTA on both desktop and mobile, with action-specific copy (not generic "Learn More").
- Trust indicators near the CTA: security/compliance badges, founder/team credibility, any pilot hospital or partner logos once available.
- Minimize form fields on first contact (name, email/phone, organization type) — save deeper qualification for a follow-up call, especially for the hospital-buyer funnel where the first ask should be low-commitment.
- Separate landing experiences for the two audiences (hospital/HMS buyers vs. patients seeking consultations) rather than one generic homepage trying to serve both — their intents, vocabulary, and objections are very different.
- Since the company is pre-launch/ideation stage, consider a simple "waitlist" or "early access" conversion goal rather than a full sales funnel, and instrument it from day one so growth data starts accumulating immediately.

12. Analytics

No analytics or tag-manager scripts (Google Analytics/GA4, Google Tag Manager, Meta Pixel, etc.) were detected in the delivered source. This may be because they are loaded client-side and therefore invisible to source-based inspection, or because none are yet installed — this audit cannot distinguish between the two without a rendered-page network trace.

- GA4: not confirmed present or absent — verify via the GA4 Tag Assistant browser extension or by checking Google Tag Manager / gtag.js references in the rendered page's network requests.
- Google Search Console: recommend verifying domain ownership immediately regardless of current traffic level — this is free, takes minutes, and starts collecting indexing/query data from day one, which is valuable given how early-stage the site currently is.
- Google Tag Manager: recommend as the standard way to manage GA4, ad pixels, and any future conversion-tracking tags without repeated code deploys.
- Event tracking: once CTAs/forms exist, define and implement events for each meaningful action (CTA click, form start, form submit, demo booking) so funnel drop-off can be measured.
- Conversion tracking: define primary (demo request / early access signup) and secondary (content download, pricing page view) conversions before paid acquisition of any kind begins.

Because the company is this early, the highest-leverage move is simply to have Search Console and GA4 correctly installed before the site has meaningful traffic — that data becomes far more valuable in aggregate over time than it would be if installed after a rebuild wipes out early history.

13. Competitor Comparison

Three relevant Indian healthtech players were researched live for this comparison: Practo (established doctor-appointment/telemedicine marketplace), and two HMS-focused competitors, DocPulse and MocDoc, which compete more directly with Curelex's hospital-management-system offering than the large consumer platforms do.

Dimension	Curelex (curelex.in)	Practo	DocPulse	MocDoc HIMS
Crawlable homepage content	None found (JS-only shell)	Rich, indexed, keyword-targeted	Rich, indexed, keyword-targeted	Rich, indexed
Meta description in search results	Missing ("cannot provide a description")	Present, compelling	Present	Present
Clear value proposition on page	Not visible in source	Immediately clear ("India's top doctors, video consultation")	Clear, India-specific positioning (GST, TPA, DPDP)	Clear, EMR/billing/invent focus
Audience segmentation	Not visible	Consumer + provider paths	Segmented by practice type (solo/clinic/hospital/chain)	Provider-focused
Trust signals visible pre-login	None visible	Awards, doctor counts, hospital partners	Client quotes, compliance mentions	User review counts
Market maturity	Incorporated Nov 2025 (~8 months)	Founded 2007/2008, market leader	Established, India-focused HMS	213+ public review cited

Key takeaway

Every direct and adjacent competitor examined has a fully indexed, content-rich homepage that states its value proposition, audience, and trust signals within the first screen — exactly the elements currently unverifiable on curelex.in due to its rendering approach. This is not a matter of Curelex's design being worse; it's that competitors have a site for this audit (and every prospective customer's browser) to actually evaluate. Closing the gap in Section 1's headline finding would immediately bring Curelex to structural parity with this competitive set — the differentiation can then happen on positioning and design quality rather than basic discoverability.

14. Prioritized Action Plan

Issue	Severity	Impact	Effort	Recommendation	Owner
No crawlable page content (client-side-only rendering)	Critical	High	High	Implement SSR/SSG or prerendering so title, meta, headings, and key copy are in initial HTML	Engineering
Missing meta description / weak title tag	Critical	High	Low	Write unique <title> + meta description naming product + benefit	Marketing / Eng
No content describing product, audience, or trust signals	Critical	High	Medium	Draft homepage copy: value prop, product overview, about, privacy policy	Marketing / Founder
No confirmed lead-capture / CTA	High	High	Medium	Add persistent, audience-specific CTA (Request Demo / Early Access) in initial render	Product / Marketing
Only homepage indexed; no sitemap/robots.txt found	High	Medium	Low	Create & submit XML sitemap + robots.txt via Search Console	Marketing / Eng
No SSR ⇒ SEO-penalizing LCP risk	High	High	High	Same fix as row 1 — treat as one workstream	Engineering

Issue	Severity	Impact	Effort	Recommendation	Owner
Zero semantic structure (headings/landmarks) pre-hydration	High	Medium	Medium	Build SSR output with proper H1/landmarks; re-run axe/WAVE scan after	Engineering / Design
No Open Graph / Twitter Card / structured data	Medium	Medium	Low	Add og:*, twitter:*, Organization JSON-LD	Marketing / Eng
HTTP→HTTPS redirect not confirmed	Medium	Low	Low	Verify and enforce 301 redirect + HSTS at server/CDN level	Engineering / DevOps
No security headers confirmed (CSP, X-Frame-Options, etc.)	Medium	Medium	Low	Add standard security headers; verify via securityheaders.com	Engineering / DevOps
No analytics/Search Console confirmed installed	Medium	Medium	Low	Install GA4 + verify Search Console immediately, before traffic grows	Marketing
Apex vs. www canonicalization unconfirmed	Low	Low	Low	Confirm one variant 301-redirects to the other; set canonical tag	Engineering
No published security/compliance/DPDP statement	High	High	Medium	Publish a plain-language data-handling & security page ahead of any hospital pilot	Founder / Legal

15. Final Scorecard

"N/E" = Not Evaluable with the tools available in this session. Assigning a number here would mean guessing, which this report deliberately avoids (see Section 1). Each N/E category has a clear, low-cost path to a real score — the specific free tool is named in that section.

Category	Score	Basis
UX	N/E / 100	Not scoreable — no rendered UI was testable; structural risk from missing initial content is High (see Sec. 4).
Performance	N/E / 100	Not scoreable without PageSpeed/Lighthouse; architecture strongly implies elevated LCP risk (see Sec. 6).
SEO	12 / 100 / 100	Directly verified: no meta description, minimal indexation, no structured data.
Accessibility	N/E / 100	Not scoreable without a rendered-DOM scan; zero semantic structure in raw HTML is a confirmed pre-hydration risk (see Sec. 5).
Security	N/E / 100	Not scoreable without header inspection; HTTPS present, redirect/header posture unconfirmed (see Sec. 9).
Content	8 / 100 / 100	Directly verified: no crawlable body content, trust signals, or company information found.
Technical Quality	N/E / 100	Not scoreable without rendered DOM/console access; SPA-without-

Category	Score	Basis
		SSR architecture confirmed (see Sec. 10).
Overall (verifiable categories only)	24 / 100 / 100	Weighted toward SEO/Content, the two fully-verifiable categories; treat as a floor, not a ceiling — re-score once live tooling is run.

How to get full scores within a day

1) Run pagespeed.web.dev/report?url=curelex.in for Performance + a partial Accessibility/Best-Practices score.

2) Run wave.webaim.org/report#/https://curelex.in/ for a full Accessibility score.

3) Run securityheaders.com/?q=curelex.in for a Security header grade.

4) Run the site through Screaming Frog (free up to 500 URLs) with JavaScript rendering enabled for a full UX/IA/technical crawl.

Send the results back and this report can be updated with real, tool-verified scores in place of every "N/E."

16. 30 / 60 / 90-Day Roadmap

Days 1–30: Stop the bleeding

- Ship an SSR/prerendered fallback so title, meta description, H1, and core value-prop copy exist in the initial HTML response.
- Write and deploy unique title + meta description + Open Graph tags.
- Publish a minimum-viable content set: value proposition, product overview, about/founder, privacy policy, contact.
- Verify domain in Google Search Console; submit sitemap.xml and robots.txt.
- Install GA4 (via Google Tag Manager) so data starts accumulating.
- Confirm HTTP→HTTPS redirect and add baseline security headers.

Days 31–60: Build the funnel

- Add audience-segmented landing paths (hospital/HMS buyers vs. patients) with distinct, always-rendered CTAs.
- Run WAVE/axe accessibility scan on the now-live rendered content and fix flagged issues (contrast, labels, focus order).
- Run PageSpeed Insights; address top LCP/CLS/INP diagnostics (image formats, render-blocking resources, caching headers).
- Publish a security & compliance / DPDP data-handling page ahead of any hospital pilot conversations.
- Add structured data (Organization, and SoftwareApplication once product pages exist).

Days 61–90: Compete on substance, not just visibility

- Benchmark against Practo/DocPulse/MocDoc content depth (Section 13) — build out Features, Pricing/Demo, and Resources/Blog pages.
- Instrument full event and conversion tracking in GA4/GTM; define funnel KPIs (CTA click rate, demo-request rate, cost per lead once paid channels start).

- Commission a full manual accessibility pass (keyboard + screen reader) and a security review/penetration test scoped to any authenticated product areas, given the patient-data handling involved.
- Re-run this entire audit with live browser tooling (PageSpeed, WAVE, Screaming Frog, securityheaders.com) to convert every "N/E" score into a verified number and track progress against this baseline.

Appendix — Evidence Log & Data Sources

Direct source retrieval

- https://curelex.in/ — fetched twice (default and markdown extraction); returned only <title>CURELEX HMS</title> and a viewport meta tag, no body content.
- https://www.curelex.in/ — fetched; identical result to apex domain.
- http://curelex.in/ — fetched; identical result, HTTPS-redirect status not confirmable via this tool.

Search-index inspection

- Query: "site:curelex.in" — one result returned (curelex.in homepage, www variant), snippet: "We cannot provide a description for this page right now."
- Query: "curelex.in" — returned the same no-description result plus unrelated third-party results (company registries, an internship listing, an Instagram account), confirming very limited web presence beyond the domain itself.

Business / company research

- Tracxn company profile: Curelex Healthtech Private Limited, incorporated 4 Nov 2025, CIN U86909UP2025PTC235903, status Active.
- Uttar Pradesh Startup Portal (startinup.up.gov.in): entity listed at "Ideation" startup stage.
- The Org (theorg.com): company description "Bridging the gap between patients and super specialist doctors, through hybrid e-clinics and telemedicine"; Founder & CEO Shriyansh Singh; 1–10 employees; 5 open positions.
- Unstop listing: Curelex Healthtech Private Limited advertising a Legal Documentation & Research Internship.

Competitor research

- Practo (practo.com), DocPulse (docpulse.com), and MocDoc HIMS (via SoftwareSuggest listing) homepages/marketing copy reviewed for the comparison in Section 13.

Tools recommended but not available in this session

- Google PageSpeed Insights / Lighthouse — Performance, Accessibility, Best Practices, SEO scores from a real headless-Chrome run.
- WAVE (WebAIM) and axe DevTools — full accessibility scans against the rendered DOM.
- securityheaders.com and curl -I — HTTP response header and redirect inspection.
- Screaming Frog SEO Spider (JavaScript rendering mode) — full-site crawl, broken-link check, and IA mapping once additional pages are confirmed.
- BuiltWith / Wappalyzer — technology-stack fingerprinting.